PLANNING BRIEF

Long COVID burden and pilot planning brief

Deterministic synthesis of reviewed national burden, analyst-supplied planning assumptions, explicitly selected implementation context, evidence gaps, and interpretation guardrails.

Brief provenance
National reviewed artifact: meps-2024-long-covid-candidate-20260926T011343Z
Reviewed local-context artifact: local-context-wv-hrsa-acs-candidate-20260924T052332Z
Generated: 2026-10-16T04:00:58.479Z

Decision state

Burden estimates	Reviewed national	**Scenario arithmetic**	Planning assumptions
Evidence parameter	No approved record loaded	**Local context**	1 selected
Data requests	1 selected		

Interpretation boundaries

- Gross burden is not attributable burden.
- Local context informs implementation and does not modify national burden.
- Scenario outputs are deterministic planning quantities, not savings, ROI, causal benefit, or observed improvement.
- Out-of-pocket/self-family-paid spending is nested within total spending and must not be added to it.
- K6 measures nonspecific psychological distress and PHQ-2 is a depressive-symptom screen; neither is a psychiatric diagnosis.
- ESAQ time measures are AHRQ annualized hours-per-year measures; later ESAQ responses do not establish continued current Long COVID after the earlier Round 3/1 status anchor.

Reviewed national burden findings

Economic / healthcare burden

13 reviewed

Measure	Estimate	Confidence interval
Represented population with operational current Long COVID	6.8M	95% CI 5.9M–7.7M
Gross annual all-cause medical spending	$97.3B	95% CI $72.0B–$122.5B

Measure	Estimate	Confidence interval
Mean annual all-cause medical spending per person	**Mean:** $14,238 **Typical Range (25th–75th percentile):** $1,116–$14,840 **Median:** $3,624	95% CI $10,668–$17,807
Out-of-pocket annual spending component	$13.5B	95% CI $6.5B–$20.5B
Mean out-of-pocket annual spending per person	$1,980	95% CI $945–$3,014
Mean annual office-based provider visits	12.4 visits/person/year	95% CI 9.4–15.4 visits/person/year
Proportion with at least one office-based provider visit	81.7%	95% CI 77.2%–86.1%
Mean annual outpatient department provider visits	2.3 visits/person/year	95% CI 1.5–3.1 visits/person/year
Proportion with at least one outpatient department provider visit	33.3%	95% CI 27.9%–38.6%
Mean annual emergency-room visits	0.4 visits/person/year	95% CI 0.3–0.5 visits/person/year
Proportion with at least one emergency-room visit	24.4%	95% CI 19.7%–29%
Mean annual hospital discharges	0.1 discharges/person/year	95% CI 0.1–0.2 discharges/person/year
Proportion with at least one hospital discharge	8.3%	95% CI 5.5%–11.1%

▼ Interpretation and technical details

Domain limitations: All spending and utilization estimates are gross/all-cause descriptive burden. The cohort is classified at the Round 3/1 2024 interview; annual 2024 spending/utilization may occur before or after that point and does not imply year-long symptom persistence. Out-of-pocket spending is nested within total spending and is not additive.

- **Represented population with operational current Long COVID:** Survey-weighted number of status-classifiable U.S. civilian noninstitutionalized adults age 18-64 meeting the documented operational Round 3/1 2024 current-Long-COVID rule.

  System boundary: National descriptive population estimate

- **Gross annual all-cause medical spending:** Calendar-year 2024 gross all-cause health care spending among adults age 18-64 who met the operational current-Long-COVID definition at their Round 3/1 interview in 2024.

  System boundary: National all-cause health care expenditures
- **Mean annual all-cause medical spending per person:** Mean calendar-year 2024 all-cause health care spending among adults age 18-64 who met the operational current-Long-COVID definition at their Round 3/1 interview in 2024.

  System boundary: National all-cause health care expenditures
- **Out-of-pocket annual spending component:** Self/family-paid component of calendar-year 2024 all-cause spending among adults classified operational-current at the Round 3/1 2024 interview.

  System boundary: National all-cause health care expenditures; source-of-payment component
- **Mean out-of-pocket annual spending per person:** Mean self/family-paid component of calendar-year 2024 spending among adults classified operational-current at the Round 3/1 2024 interview.

  System boundary: National all-cause health care expenditures; source-of-payment component
- **Mean annual office-based provider visits:** Mean calendar-year 2024 all-cause office-based provider visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one office-based provider visit:** Share with at least one office-based provider visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual outpatient department provider visits:** Mean calendar-year 2024 all-cause outpatient department provider visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one outpatient department provider visit:** Share with at least one outpatient department provider visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual emergency-room visits:** Mean calendar-year 2024 all-cause emergency-room visits among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one emergency-room visit:** Share with at least one emergency-room visit during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization
- **Mean annual hospital discharges:** Mean calendar-year 2024 all-cause hospital discharges among adults classified operational-current at their Round 3/1 interview in 2024.

  System boundary: National annual all-cause health care utilization
- **Proportion with at least one hospital discharge:** Share with at least one hospital discharge during calendar-year 2024 among adults classified operational-current at their Round 3/1 2024 interview.

  System boundary: National annual all-cause health care utilization

Healthcare-work / administrative burden

4 reviewed

Measure	Estimate	Confidence interval
Annualized time on own medical visits / filling prescriptions	76.5 hours/person/year	95% CI 45.7–107.2 hours/person/year
Annualized time managing medical bills / insurance	27.5 hours/person/year	95% CI 13.5–41.5 hours/person/year
Annualized health care paperwork / navigation time	**Mean:** 24.5 hours/person/year **Typical Range (25th–75th percentile):** 0 hours/person/year–12 hours/person/year **Median:** 3 hours/person/year	95% CI 12.8–36.2 hours/person/year
Prior approval denied or delayed — yes/no response universe	34.3%	95% CI 27.2%–41.5%

▼ **Interpretation and technical details**

Domain limitations: ESAQ native time domains are reported separately and are not summed. ESAQ respondents were classified operational-current at the earlier Round 3/1 2024 anchor; later ESAQ responses do not prove continued current status. ESAQ time measures are top-coded by AHRQ and have item-response limitations.

- **Annualized time on own medical visits / filling prescriptions:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for own medical visits and filling prescriptions among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Annualized time managing medical bills / insurance:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for paying or managing medical bills and insurance among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Annualized health care paperwork / navigation time:** Survey-weighted mean of the AHRQ annualized hours-per-year measure for paperwork, finding doctors, plan information, and approvals among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 who had been classified operational-current at the earlier Round 3/1 2024 interview.

  System boundary: National healthcare-work / administrative burden; ESAQ native time domain
- **Prior approval denied or delayed — yes/no response universe:** Proportion reporting that health insurance denied or delayed prior approval among later ESAQ respondents in Panel 28 Round 5 / Panel 29 Round 3 previously classified operational-current at the earlier Round 3/1 2024 interview who supplied a substantive Yes/No response.

  System boundary: National ESAQ prior-approval experience

Mental-health / human burden

4 reviewed

Measure	Estimate	Confidence interval
Mean K6 nonspecific psychological distress score	5.9 score (0-24)	95% CI 5–6.8 score (0-24)
Serious nonspecific psychological distress (K6 >=13)	14.1%	95% CI 9.3%–18.8%
Mean PHQ-2 depressive-symptom screen score	1.3 score (0-6)	95% CI 1.1–1.6 score (0-6)
PHQ-2 screen-positive (score >=3)	20.6%	95% CI 14.7%–26.4%

▼ **Interpretation and technical details**

Domain limitations: K6 measures nonspecific psychological distress; PHQ-2 is a depressive-symptom screen. PSAQ respondents were classified operational-current at the earlier Round 3/1 2024 anchor; later PSAQ outcomes do not prove continued current status. Neither is a psychiatric diagnosis, monetized burden, or causal Long COVID effect.

- **Mean K6 nonspecific psychological distress score:** Mean official K6 nonspecific psychological distress score among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ nonspecific psychological distress
- **Serious nonspecific psychological distress (K6 >=13):** Proportion meeting AHRQ's published K6 >=13 serious-distress category among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ nonspecific psychological distress
- **Mean PHQ-2 depressive-symptom screen score:** Mean official PHQ-2 depressive-symptom screening score among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ depressive-symptom screening
- **PHQ-2 screen-positive (score >=3):** Proportion with PHQ-2 score >=3 among later PSAQ respondents previously classified operational-current at the Round 3/1 2024 anchor.

  System boundary: National PSAQ depressive-symptom screening

Pilot planning assumptions

Illustrative planning defaults — edit before use These inputs are planning assumptions, not observed evidence.

Potentially eligible	**12,000**	Intended reach	**25% (0.25)**
Program capacity	**2,000**	Baseline recognition window	**12 months**
Planning recognition window	**6 months**	Planning horizon	**12 months**
Fixed program cost	**$300,000**	Cost per participant	**$300**

MODELED PATHWAY STEP
Recognition → referral

ASSUMED MECHANISM
Navigation support

Reach, capacity, recognition-window assumptions, and program-resource inputs apply only within this modeled step and mechanism.

Outside this model: Reviewed national burden estimates; Unrelated care-pathway stages; Treatment effectiveness; Clinical outcomes.

Scenario outputs

Deterministic planning quantities from the applied assumptions above; these are not a forecast or observed evidence.

Participants within capacity	**2,000**	Person-month planning exposure difference	**12,000**
Program-resource requirement	**$900,000**		

Decision trace

Evidence → explicit analyst context → planning scope → evidence/data gap. This is a provenance and decision ledger, not a proven causal chain.

01 **Released observed burden**
Reviewed national MEPS burden · 21 reviewed estimates · national descriptive evidence

02 **Explicitly selected reviewed local constraint/context**
Distinct currently Designated Primary Care HPSA IDs: 123 · West Virginia state context

03 **Target pathway step**
Recognition → referral

04 **Planning mechanism**
Navigation support

05 **Measurement / data request needed**
1 current operational data request

06 **Supported claim boundary**
No reviewed intervention-effect parameter is available. The scenario remains assumption-based; causal benefit, savings, ROI, and preventable-cost claims remain unavailable.

Local implementation context

Reviewed local-context artifact: local-context-wv-hrsa-acs-candidate-20260924T052332Z

Distinct currently Designated Primary Care HPSA IDs **Available**

West Virginia (state) **Active Pilot planning context**

123

Source: HRSA · Current HRSA HPSA snapshot retrieved 2026-09-24

Published uncertainty: Not applicable / not published

Availability: Available

Allowed interpretation: Implementation/access context for West Virginia.

Not supported: West Virginia Long COVID prevalence; West Virginia Long COVID cost; state-specific Long COVID burden; causal diagnostic-delay effect; causal pilot benefit; causal effect modifier; calibrated access severity score; attributable cost; savings; ROI

Limitations: HPSA IDs may contain multiple component rows; counts use distinct HPSA IDs. Designation counts are contextual descriptors and are not a calibrated shortage-severity scale. HPSA Score is a source-published designation priority/shortage score, not a calibrated statewide access-severity composite. No HPSA designation populations, FTE-short values, or other overlapping quantities are summed.

Intervention-effect evidence

No reviewed intervention-effect parameter is loaded.

Causal benefit, savings, ROI, and preventable-cost claims remain unavailable.

What to collect next

Only requests explicitly selected by the analyst are included.

- **What provider/program capacity is actually available for this pilot's modeled pathway step?** — staffed program capacity, available appointment or navigation slots, specialty/service type, catchment geography, wait time, staffing FTE, capacity time window

  Pathway: Recognition → referral · Mechanism: Navigation support